



Exploring concept creep: Youth's portrayal of ADHD on TikTok

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ABSTRACT

TikTok has quickly become a popular platform for youth to access and share mental health information. Videos discussing characteristics of diagnoses, such as Attention Deficit Hyperactivity Disorder (ADHD), receive significant engagement. However, concerns have been raised about the spread of misinformation and potential self-diagnosis among viewers of these videos.

To assess the accuracy of the information shared, we conducted a content analysis of 100 popular TikTok videos. Our findings showed that 55 % of the characteristics that youth attributed to ADHD in their videos did not align with the DSM-5-TR criteria of this diagnosis. Investigation of the top comments further suggests that the majority of the viewers (59,6 %) identified with the depicted behaviors and experiences. Moreover, most of the top-commenters accepted or even validated that these traits were part of ADHD, even though this was often not the case.

Our findings illustrate how youth broaden the ADHD concept on TikTok. This bottom-up psychiatrization may affect the self-image of other young people who identify with this content, as they come to understand their own behaviors through a pathological lens. It additionally raises concerns about the already high demand for health care, as individuals may seek help after self-diagnosing. This study highlights the need to make health care providers, educators and parents aware of the misinformation circulating online about ADHD. This would enable them to better support youth and help them to be critical of mental health content they encounter on social media.

Recent figures show that on average 8 % of the children and adolescents worldwide meet the criteria for ADHD (Xu et al., 2018; Ayano et al., 2023). In the latest version of the Diagnostic and Statistical Manual of Mental Disorders (DSM-5-TR), ADHD is defined as a persistent pattern of inattention and/or hyperactivity-impulsivity, which interferes with functioning or development (American Psychiatric Association, 2022). According to this diagnostic manual, children must display at least six of the nine listed behaviors of either inattention or hyperactivity-impulsivity to qualify for diagnosis. These behaviors should be present before the age of 12 and in at least two settings (APA, 2022). ADHD has long been considered a childhood disorder; however, changes in the criteria now allow for retrospective diagnoses in adults. Despite being classified as a neurodevelopmental disorder in dominant discourse, including by the APA, studies investigating biomarkers of ADHD have not provided conclusive evidence, and are unlikely to do so due to the diagnosis' heterogeneity (Schleim, 2022; Samea et al., 2019). Given that the diagnosis relies on behavioral checklists, ADHD therefore remains a descriptive classification (Kirk et al., 2015). From this

viewpoint, ADHD is best understood as a social construct, as its definition is shaped by cultural norms and expectations, rather than biological tests (Timimi and Taylor, 2004).

Inattention and hyperactivity were first labeled as ADD (later ADHD) in the third edition of the DSM (APA, 1980). Since then, the diagnostic criteria for ADHD have been broadened with almost every revision of the manual. Haslam (2016) has described the expansion of psychological classifications with the terms horizontal and vertical concept creep. Horizontal concept creep occurs when new criteria are added that are qualitatively different from the original characteristics, such as when new impulsivity criteria were introduced in DSM-III-TR (Newcorn et al., 1989). Vertical concept creep indicates that current diagnostic thresholds are lowered to include less severe phenomena, which was the case when the requirement for adult diagnosis was reduced from six to five criteria (Abdelnour et al., 2022). The repeated expansion of the ADHD criteria in both directions partly explains why the group who fits into this diagnostic category has continued to grow (Fabiano and Haslam, 2020; Batstra and Frances, 2012a). While some regard this rise in

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diagnostic rates as a sign of improved detection of those who are suffering and in need of (medical) support (Abdelnour et al., 2022), others warn for overdiagnosis, arguing that for additional detected cases on the milder end of the spectrum the potential benefits of a diagnosis may not outweigh its harms (Kazda et al., 2021).

As a result of concept creep, new or milder behaviors are understood through a psychiatric lens. According to the framework of Beeker et al. (2021), this psychiatrization not only occurs from the top-down but is also frequently driven by lay demands. Youth indicate that a diagnosis can exempt them from blame and responsibility, offering an explanation for their actions, and can therefore be used to gain social acceptance for their deviant behavior (Honkasilta et al., 2016). Although ADHD is formally a descriptive diagnosis, the term is frequently reified into a medical entity that is seen as the cause of behavior (Te Meerman et al., 2022). This biomedical conception of ADHD is reinforced by the common belief that ADHD is a brain disorder, even though small observed brain differences at the group level do not apply to most children with ADHD and environmental factors also influence behavior (Batstra et al., 2014). The tendency to treat ADHD as a disease with specific ‘symptoms’, rather than as a description with set criteria, makes it susceptible to concept creep. The ADHD label can provide a sense of relief and understanding, both for those who receive the diagnosis and those around them (Werkhoven et al., 2022). This can stimulate people to attribute other, previously undiagnosed, behaviors to ADHD, which they assume might also be a sign of this underlying ‘cause’.

With the rise of the Internet, struggling individuals no longer need to rely on professionals to acknowledge and classify their mental health issues, but increasingly turn to social media (Lee, 2022). The platform TikTok is particularly popular among young people. In 2021, TikTok reached the milestone of 1 billion active users, with more than half of its users under the age of 25 (TikTok, 2021; Statista, 2024). Although its original intended purpose was to entertain, youth increasingly turn to the platform to share and seek information on mental health in short-, fast-paced videos (Milton et al., 2023; Aragon-Guevara et al., 2023). Diagnostic labels are ‘trending’, with Attention Deficit Hyperactivity Disorder (ADHD) among the most popular health-related hashtags used on TikTok, resulting in billions of hits (Zenone and Barbic, 2021). Videos with titles such as ‘if you show these 5 signs you might have ADHD’ are suggested to encourage self-diagnosis among youth (Caron, 2022; Kelly, 2023). As such, TikTok may serve as an important medium through which psychological classifications are disseminated amongst adolescents and young adults, allowing youth to self-identify with a diagnosis without having to meet its clinical criteria (Gilmore et al., 2022; Haltigan et al., 2023). The anticipated validation and understanding offered by a diagnosis may encourage video creators to cite ADHD as the source of their undesirable behavior, even if this behavior does not (yet) fit the clinical criteria, thereby expanding the definition of this diagnostic category. This raises concerns about the reliability of the information that youth share about ADHD on this platform.

TikTok currently does not have a system in place to fact-check mental health information shared on its platform (Aragon-Guevara et al., 2023). This is alarming, given that most videos are created by young, non-professional users (Basch et al., 2022). Initial research suggests that approximately half of the information about ADHD that is shared on TikTok is considered incorrect by clinical professionals (Yeung et al., 2022; Karasavva et al., 2025). The operating mechanisms of social media are suggested to facilitate this spread of misinformation. TikTok’s algorithm will present users with videos that are similar to those they have previously engaged with, potentially leading them into an echo chamber of misinformation (Cinelli et al., 2021). This curation is often done so effectively that it has been suggested to alter young users’ self-concept, as the highly personalized content gives the impression that TikTok knows some hidden parts of their identity that they have not yet discovered for themselves (Milton et al., 2023). As social media feed into adolescents’ heightened sensitivity to social comparison and peer feedback during a critical stage in their identity formation, the

misinformation spread on these platforms could lead to increased self-identification with an already wide spread diagnosis (Peters et al., 2021).

Although it has been suggested that social media may normalize mental health issues (such as inattention and hyperactivity) by increasing their visibility, posts might have an opposite effect if they simultaneously pathologize these issues and the people who experience them (Hasan et al., 2023). Diagnosed individuals often face stigmatization, and are perceived as less competent, more antisocial and more dangerous than people without a label displaying the same behavior (Mueller et al., 2012; Ohan et al., 2013; Lebowitz, 2016). While self-diagnosed individuals will not face discrimination as long as they keep their diagnosis private, they may apply these internalized negative stereotypes to themselves (Moses, 2009; Nguyen and Hinshaw, 2020). Internalized stigma has been found to negatively impact young people’s self-image, resulting in lower self-esteem (Harari et al., 2023; Livingston and Boyd, 2010). Biogenetic explanations of psychological problems, which diagnoses are often thought to provide, can instill the belief in diagnosed individuals that they are inherently different and unable to change. This can cause pessimism and feelings of hopelessness, which can turn into a self-fulfilling prophecy (Ching and Ma, 2021; Kvaale et al., 2013; Hennig et al., 2023). At the same time, broadening the ADHD concept may encourage people to seek professional care, which has been suggested to benefit currently underserved populations who otherwise may not seek help (Haslam et al., 2021). However, this also presents challenges for the public health system in the Netherlands, which is currently already overburdened. General practitioners are inclined to take immediate action when youth enter their practice (van Dijk et al., 2022). If the boundaries of ADHD are extended to include relatively mild behavioral issues, there lies a risk that limited resources are diverted away from those with more severe or complex needs (Beeker et al., 2021).

The current study explores to what extent the characteristics that youth attribute to ADHD on TikTok are in line with the DSM-5-TR criteria of this diagnosis. It is hypothesized that the perceived benefits of a diagnosis, combined with the desire for likes and views, may encourage young people to expand the ADHD concept beyond its clinical boundaries in their TikTok videos, thereby contributing to the spread of misinformation. Given that these videos have been suggested to contribute to self-diagnosis, we also examine whether viewers self-identify with this content, and assess how their self-image may be impacted by the diagnostic claims made in these videos.

1. Method

1.1. Data collection

TikTok’s ‘For You’ discovery page was searched on May 10, 2023 using the hashtag #ADHD. The search returned 24.7 billion hits. Videos on the ‘For you’-page are not presented chronologically, but rather based on an algorithm that takes into account the popularity of the videos, as well as the user’s location and viewing history (Herrick et al., 2021). A new account was created for the search, to prevent previous engagement from influencing the results. The first 100 videos that met the inclusion criteria were collected. A video was included if it depicted behaviors or experiences which were attributed to ADHD. Videos were excluded if the estimated age of the creator was over 30, or if the creator mentioned any health care or research affiliations. In total 11 videos were excluded from the study based on these criteria. Links to eligible videos were temporarily saved. As consent for long-term storage was not obtained, the videos were immediately transcribed without any identifiable data, after which the links to the videos were deleted. Views per video at time of data collection ranged from 37,200 to 19.3 million, the number of likes from 3009 to 3.5 million and the number of comments from 121 to 28,900. In total, the included videos received 316.1 million views, 47.7 million likes and 553,002 comments.

The top three comments from each of the 100 videos were also collected, resulting in a total of 297 comments (as the comment option was deactivated for one video). As with the videos, TikTok sorts the comments using an algorithm, which typically results in the comments with the most previous engagement appearing first. The most popular comments were selected, as their high engagement in terms of likes was seen as an indication that their content resonated with a large audience. Moreover, popular comments are often more substantive in terms of content, which makes them more suitable for content analysis (Risch and Krestel, 2020; Berry and Taylor, 2017). Comments received a total of 2.3 million likes and 10,713 replies, with the number of likes per comment ranging from 0 to 313,500 ($M = 7785$) and the number of replies from 0 to 1936 ($M = 36$). The study was approved by the Ethics Committee of the Department of Pedagogical and Educational Sciences of the University of Groningen, The Netherlands (PED-2223-S-0044).

1.2. Transcription

As most TikTok videos used both audio, visuals and/or text to depict ADHD characteristics, we opted for a multimodal transcription method. Large variety exists in multimodal transcription practices, as the relevance of each aspect of the recording is highly dependent on the research aim. We developed a new transcription scheme to capture elements relevant to this study, including text, spoken audio, background audio, nonverbal actions and setting (Mondada, 2018). Spoken audio and text were transcribed for all videos, other modes were transcribed only when they provided information about the portrayed ADHD characteristics. An adapted version of Jefferson’s annotation scheme was used to capture the intonation and speed of spoken text where relevant (See Appendix A; 2008; Sacks et al., 1974). Nonverbal actions that were transcribed include gestures, body position and movement and facial expressions. Finally, for the setting we reported on the surroundings, objects and clothing shown in the video, if relevant. The first two authors both transcribed the same 20 videos and discussed their findings to reach agreement on which elements should be transcribed. The first author then transcribed the remaining 80 videos. To protect the privacy of the video creators, no literal transcripts are displayed in the results. Instead, video transcripts and comments were paraphrased or merged into fictional examples to illustrate the original data.

1.3. Data analysis

After obtaining an initial impression of the videos, qualitative content analysis was employed to examine the portrayed characteristics of ADHD. First, we used deductive coding to identify behaviors that corresponded with DSM-5-TR criteria of ADHD. A conservative approach was adopted, by coding behaviors using the DSM criteria whenever interpretation allowed. Other behaviors were coded inductively, using multiple feedback loops. Ten randomly selected transcripts were coded by the first two authors to reach agreement on inductive codes and develop decision rules in case DSM-5-TR criteria overlapped. Next, twenty transcripts were double coded, resulting in good reliability (Krippendorff’s binary kappa = 0.844, Cu-kappa = 0.795). The remaining transcripts were coded by the first author. Subsequently, comments were analyzed to examine if viewers self-identified with the content of the videos. Additionally, it was examined if commenters believed that the depicted behaviors and experiences were indeed characteristics of ADHD, as was claimed in the videos. Based on preliminary investigation by the first two authors, comments were coded inductively as either *accepting*, *validating*, *questioning* or *rejecting* these claims. A subset of 60 comments (stemming from 20 videos) were coded by both authors using these categories, resulting in an agreement of Cu-kappa = 0.889.

2. Results

2.1. General description videos

2.1.1. Video type and content

Included videos varied in their mode of communication (Li et al., 2021). In the majority of the videos ($n = 62$), ADHD characteristics were acted out, either by an individual creator or in the form of a role play. These videos often had a humorous undertone. Other creators ($n = 28$) presented the information via speech, often in a more formal manner. Finally, some creators ($n = 6$) chose to share short clips of their real-life experiences with ADHD, while others used animations ($n = 3$) or performed a dance while the ADHD characteristics only appeared in text ($n = 1$). The diagnostic claims that creators made were either general or personal. In the majority of the videos ($n = 80$), the creators made general claims regarding the characteristics of ADHD: they mentioned how the described behaviors were ‘signs’ or ‘symptoms’ of ADHD, acted out how ‘someone with ADHD’ acts or feels (often in comparison to someone without ADHD) or described ‘what ADHD looks or feels like’. Others acted out how ‘the ADHD brain’ works, or made statements in which ADHD was the implied cause of behavior, such as: ‘ADHD makes it difficult to regulate emotions’. In the personal videos ($n = 20$), creators talked specifically about the characteristics of ‘their (life with) ADHD’ or about the symptoms they themselves or their partner experienced as a person with ADHD.

2.2. Correspondence between depicted characteristics and the DSM-5-TR criteria of ADHD

Video creators attributed a total of 411 distinct characteristics to ADHD in their videos. Of these, 186 (45 %) corresponded with the DSM-5-TR criteria of ADHD, while 225 (55 %) did not. In only 21 of the 100 videos, all the mentioned characteristics corresponded with the DSM-5-TR criteria of ADHD. Most often ($n = 52$), creators mentioned a mix of DSM and non-DSM characteristics in their videos, while in 27 videos none of the behaviors corresponded with the DSM-5-TR criteria of ADHD. Table 1 displays in how many videos each of the behaviors listed in the DSM-5-TR criteria was mentioned.

Table 1
Prevalence of depicted behaviors that correspond with the DSM-5-TR criteria of ADHD.

Inattention Criteria	N	Hyperactivity and Impulsivity Criteria	N
1a. Fails to give close attention to detail or makes careless mistakes	2	2a. Fidgets with or taps hands or feet or squirms in seat	10
1b. Has difficulty sustaining attention in tasks or play activities	23	2b. Leaves seat in situations when remaining seated is expected	0
1c. Does not seem to listen when spoken to directly	6	2c. Runs about or climbs in situations where it is inappropriate	2
1d. Does not follow through on instructions and fails to finish schoolwork, chores or duties in workplace	11	2d. Unable to play or engage in leisure activities quietly	2
1e. Has difficulty organizing tasks or activities	28	2e. Is often ‘on the go’ acting as if ‘driven by a motor’	10
1f. Avoids, dislikes or reluctant to engage in tasks that required sustained mental effort	5	2f. Talks excessively	8
1g. Loses things necessary for tasks or activities	5	2g. Blurts out an answer before a question has been completed	1
1h. Easily distracted by extraneous stimuli	21	2h. Has difficulty waiting his or her turn	3
1i. Forgetful in daily activities	22	2i. Interrupts or intrudes on others	2

Note. N represents the amount of TikTok videos in which the behaviors were depicted.

2.2.1. Characteristics congruent with DSM-5-TR criteria

Inattentive behaviors. Of the 186 characteristics that were coded with DSM-5-TR criteria of ADHD, the majority (75 %) exemplified inattention (see Table 1). In their videos, youth most frequently mentioned having difficulties with organizing tasks and activities as a characteristic of ADHD (1e), which was illustrated by them having difficulty managing time and cleaning up after themselves. ADHD was additionally related to an all-or-nothing mentality, where people do either everything at once, or nothing at all, reflecting a difficulty managing sequential tasks which is also part of criterium 1e. In line with criterion 1b, video creators also often addressed that having a short attention span is part of ADHD, illustrated by ‘zoning out’ during lectures or conversations. Additionally, people with ADHD were shown to easily get distracted by their thoughts or surroundings (1h). These videos were often shot from the point of view of someone with ADHD (see Appendix B-1 for a fictional example). Other videos displayed a person with ADHD repeatedly changing the topic mid-conversation to a seemingly unrelated subject, which similarly illustrates how they get distracted by unrelated thoughts.

Youth additionally stated in their videos that forgetfulness is a characteristic of ADHD, which corresponds with criterion 1i. However, the examples given in the DSM-5-TR refer to forgetting tasks or appointments, whereas in the videos it was shown how people with ADHD forget to eat and drink and have difficulty retaining factual knowledge. Other forms of inattention listed in the DSM-5-TR were not mentioned as often by video creators, although some did illustrate that having difficulty listening (1c), failing to finish tasks such as cleaning (1d), avoiding complicated tasks (1f) and losing one’s phone, wallet or keys (1g) are part of ADHD.

Hyperactive and impulsive behaviors. Hyperactive and impulsive behaviors, as outlined in the DSM-5-TR criteria, were not as frequently addressed by youth in their TikTok videos as the inattentive behaviors. Only 25 % of the deductively coded fragments met the criteria for hyperactivity and impulsivity. These fragments mainly depicted behaviors included in criterion 2a: fidgeting with hands, rubbing feet and bouncing legs. Additionally, it was shown how people with ADHD have difficulty sitting or standing still, or experience sudden bursts of energy, as described in criterion 2e. People with ADHD were also depicted as talking excessively (2f), without giving their conversation partner time to respond. The remaining DSM-5-TR examples (2c, 2d, 2g-2i) of hyperactive-impulsive behaviors were mentioned only sporadically across all videos. None of the videos mentioned that leaving your seat when you are expected to remain seated (2b) is part of ADHD.

2.2.2. Characteristics incongruent with DSM-5-TR criteria

Behaviors and experiences that TikTok creators attributed to ADHD, but did not meet DSM-5-TR criteria, were coded inductively. These codes were subsequently organized under six main categories: cognitive processing, behavior regulation, focus, mood regulation, social interaction and creativity. Their prevalence is shown in Table 2.

Cognitive processing. Many of the youth-proclaimed characteristics of ADHD unaligned with DSM-5-TR criteria were related to cognitive processing. Foremost, TikTok creators claimed that people with ADHD often feel completely overwhelmed by their thoughts (n = 17). Some youth referred to this as ADHD- or task paralysis, since this feeling is often suggested to lead to inactivity: ‘When you have ADHD, your brain is always going, the thoughts are never ending. There is so much that needs to be done, but you can’t move, because you’re completely overwhelmed’ (fictional example compiled from several real transcripts).

Video makers additionally indicated that hyper- and hyporeactivity to sensory input (n = 15) is a characteristic of ADHD, which they suggested leads to respectively over- and understimulation. Other aspects of ADHD mentioned include ‘time blindness’ (n = 5), which refers to an unawareness of the passage of time, as well as ‘executive dysfunctioning’ (n = 4), a general term used for cognitive processing issues. Moreover, people with ADHD were claimed to be indecisive (n = 3) and have

Table 2

Prevalence of Depicted Characteristics Incongruent with the DSM-5-TR criteria of ADHD.

Main category	Characteristic	N
Cognitive processing	Feeling overwhelmed by thoughts or tasks	17
	Hyper- or hyporeactivity to sensory input	15
	Distorted sense of time	5
	Executive dysfunction	4
	Being intellectually competent	3
	Difficulty understanding or verbalizing own thoughts and feelings	3
	Indecisiveness	3
	Having realistic dreams	1
	Making random bodily movements or sounds	14
Behavior regulation	Displaying extremes in eating behavior	4
	Displaying repetitive behavior	4
	Impulsive spending and giving	4
	Difficulty regulating substance use	3
	Displaying hyper- or insomnia	3
Focus	Experiencing hyperfocus	17
	Getting bored by repetitive tasks or activities	11
Mood regulation	Difficulty regulating own emotions	19
	Feeling anxious	7
	Feeling depressed	5
Social interaction	Rejection sensitivity	10
	Intolerant to people invading personal space	7
	Being charismatic	6
	Coming across as rude	5
	Coming across as distant	5
	Masking	2
	Annoying others	1
Creativity	Being creative	3
	Being unique	2

Note. N represents the number of videos in which these characteristics were mentioned.

difficulty understanding and verbalizing their own thoughts and feelings (n = 3). At the same time, they were also portrayed as intellectually competent (n = 3): they are considered to have brilliant ideas and be good problem solvers.

Behavior regulation. Even though youth only scarcely addressed DSM-5-TR examples of hyperactivity and impulsivity in their TikTok videos, they did attribute other forms of behavior dysregulation to ADHD. In 14 videos, creators suggested that people with ADHD often move their bodies randomly (see Appendix B-2 for a fictional example). Another depicted example of impulsive behavior that was attributed to ADHD but does not meet the DSM-5-TR criteria is impulsive spending and gift giving (n = 4). Moreover, some video creators claimed that extremes in eating (n = 4) and sleeping behavior (n = 3) are part of ADHD, as well as difficulties with regulating substance use (n = 3). People with ADHD were said to both overeat or not eat at all, and were suggested to experience both hyper- and insomnia. Finally, in four videos, the creators claimed that people with ADHD display repetitive behavior. This included both body-focused behavior, such as skin picking and chewing, as well as repeating activities and sticking to routines.

Focus. While the DSM-5-TR criteria of ADHD solely include examples of inattention, video creators also frequently attributed extreme forms of attention to ADHD (n = 17). According to the creators, an activity can grab your full attention if you have ADHD, making you ignore everything else. They refer to this as ‘hyperfocus’ or ‘hyperfixation’. After this period of hyperfocus, people with ADHD are however said to suddenly completely abandon the activity, and will have difficulty redirecting their focus. One video maker suggested that this alternation of periods of hyperfocus and complete inattention shows that ADHD does not reflect a lack of attention, which the name attention deficit hyperactivity disorder suggests, but rather a difficulty regulating it. Moreover, these shifts in attention are implied to be caused by shifts in their interest. According to some videos, people with ADHD quickly get bored by repetitive tasks and activities due to their high need for novelty

(n = 11). Creators explain: *'If you have ADHD you have many hobbies that you initially enjoy and hyperfixate on, but then one day you suddenly abandon them and seek something new.'*

Mood. In line with the mentioned behavior and attention regulation issues, youth also frequently attributed emotion regulation difficulties to ADHD (n = 17). According to them, people with ADHD often experience intense mood swings (fictional example): *'they first feel extremely happy, confident and energetic, and then suddenly feel extremely miserable or angry'*. This extreme anger, also referred to as 'ADHD rage', often seems to be directed at their loved ones. To a lesser degree, video creators attributed mere feelings of anxiety (n = 7) and depression (n = 5) to ADHD.

Social interaction. Youth also indicated that people with ADHD experience difficulties in social interaction. They mention that they often feel that they are not good enough and are sensitive to rejection, which was sometimes referred to as Rejection Sensitive Dysphoria (n = 10). Video creators additionally suggested that people with ADHD find it uncomfortable when others invade their personal space (n = 7) and can come across as distant (n = 5) or rude (n = 5). At the same time, other creators suggested in their videos that people with ADHD are charismatic (n = 6), as they refer to them as funny and empathic. Masking was additionally related to ADHD in two videos, as youth suggest that ADHD causes people to mimic the behavior of others.

Creativity. Besides charisma and intellectual competence, youth also occasionally assigned other positive personality traits to ADHD in their videos. They indicate that people with ADHD are creative (n = 3) and have a unique outlook on the world (n = 2).

2.3. Additional characteristics attributed to ADHD in the comments

In reaction to the videos, some commenters (n = 31) also attributed behaviors to ADHD, other than those mentioned in the video they commented on. In reaction to a video in which the need for novelty was discussed a commenter for example reacted: *'this, but then also being overwhelmed due to the tiniest change'*. Of the 40 behaviors that commenters attributed to ADHD, in total 24 (60 %) corresponded with the DSM-5-TR criteria, of which 13 corresponded with the criteria of inattention and the other 11 with those of impulsivity-hyperactivity. Youth most frequently mentioned behavior that corresponded with criterion 1d: difficultly following through on instructions (n = 4) and criterion 2h: difficulty waiting your turn (n = 6). Of the 16 (40 %) behaviors that did not match the DSM-5-TR criteria, most (n = 12) overlapped with the incongruent behaviors mentioned in the videos, including hyper- or insomnia (3), feeling overwhelmed by thoughts or tasks (2) and displaying repetitive behavior (2). Newly attributed behaviors included disliking (inadequate) instructions (2), an underreaction to caffeine (1) and disliking unsocial behavior (1).

2.4. Viewers' reactions to TikTok videos about ADHD characteristics

2.4.1. Self-identification

The top three comments of each video were included in the analysis, resulting in a dataset of 297 comments, as one video creator deactivated the option to comment. In more than half (59,6 %) of the comments, viewers indicated that they recognized the behaviors and experiences that were attributed to ADHD in the TikTok videos, because they themselves had experienced them. They used phrases such as *'that's so me'* or *'this sums me up perfectly'* to indicate that they related to the content, or shared their own similar experiences in the comments. Only three commenters (1,0 %) explicitly mentioned that they did not recognize the behavior displayed in the video, while they did have an ADHD diagnosis. In the remaining comments (39,4 %), the viewer did not mention whether or not they recognized the behavior or experiences in the video.

2.4.2. Diagnostic attributions

Commenters varied in their acceptance of the diagnostic claims that were made in the videos. In total, 118 viewers (39,7 %) commented on whether or not they believed the depicted behaviors and experiences were indeed characteristics of ADHD, as was said in the videos. Commenters either accepted, validated, questioned or rejected these diagnostic claims.

Of the viewers who commented on the diagnostic claims, many (55,1 %) validated the information in the videos. These commenters mentioned that the videos were *'true'* or complimented the creators on their accuracy: *'This hit the nail on the head, well explained!'*. Some commenters (n = 14) explicitly mentioned that they had been diagnosed to further support the claims made: *'As someone who actually has ADHD, this is very accurate'*. Others offered an explanation for the depicted behaviors to validate the content, for example by explaining why people with ADHD zone out. Another group of commenters did not validate the content, but was *accepting* (21,1 %) of the information shared in the videos. This was reflected in them applying the diagnosis to themselves in response to the video: *'This is exactly me, I think I have ADHD'*. Two commenters explicitly indicated that they felt they needed to take an ADHD test after watching the video. Commenters seemed to believe that the diagnosis offers an explanation for their (unwanted) behavior: *'I didn't realize this was part of ADHD, I wasn't just crazy?'*.

Only a small group of viewers was skeptical of the diagnostic claims. Four individuals (3,4 %) expressed *uncertainty* about the diagnostic claims made in the videos: *'Does this mean I have ADHD, or is this a normal thing that everyone does?'*. Other viewers (20,3 %) who commented on the diagnostic attributions more firmly *rejected* the claims. Individuals mentioned that they had experienced the depicted behavior, but denied that they had ADHD: *'I do all these things, but don't have ADHD'*. Other critical commenters actively warned their peers, stating: *'if you relate to this, that doesn't mean you have ADHD!'* They criticized the creators of the videos, mentioning that *'not everything is ADHD'*, and questioning their credibility as non-medical professionals.

3. Conclusion

The current study examined if concept creep of ADHD occurs on the social media platform TikTok. Analysis of the 100 most popular videos showed that adolescents and young adults broaden the boundaries of this concept by attributing characteristics to ADHD that do not correspond with its diagnostic criteria, exemplifying horizontal concept creep. Consistent with findings from Karasavva et al. (2025), only 45 % of the behaviors that youth attributed to ADHD align with its DSM-5-TR criteria, while more than half (55 %) did not. Most videos (52 %) contained partial misinformation, while 27 % of them were entirely misleading. Only 21 % exclusively depicted characteristics of ADHD that were in line with the DSM-5-TR. Analysis of the top three comments of each video suggests that this widespread misinformation may impact users' self-concept. Over half (59,6 %) of the commenters identified with the video content. Moreover, undiagnosed individuals generally accepted the diagnostic claims made in these videos, which were often validated by other individuals in the comments. These findings indicate that by watching TikTok videos, young viewers may come to understand their own previously considered normal behavior through the pathological lens of a diagnosis.

4. Discussion

The behaviors that youth attributed to ADHD in their TikTok videos aligning with the DSM-5-TR criteria, were predominantly related to inattention. This finding may be explained by the higher prevalence of the inattentive subtype of ADHD among adolescents (Salari et al., 2023). Other depicted behaviors were considered qualitatively different from the DSM-5-TR criteria, and therefore seen as an indication of horizontal concept creep. However, they were often still closely related to ADHD's

core categories. This suggests that youth have a general understanding of the ADHD concept, but might not be familiar with the specific criteria that define this diagnosis. This may partly be due to the diagnosis' ambiguity, as the DSM-5-TR criteria lack specificity and tend to overlap. Moreover, ADHD is often associated with behaviors outside its clinical criteria, both in research and practice. In DSM-5-TR, emotion dysregulation is for example mentioned as an 'associated feature of ADHD', which may explain why youth frequently attributed this issue to the diagnosis (APA, 2022). ADHD's high co-morbidity with other diagnoses seems to further fuel concept creep. Youth frequently attributed behaviors to ADHD that relate to autism -such as hyper- or hyporeactivity to sensory input – along with behaviors characteristic of anxiety and depression - including hyper- and insomnia.

Video creators also attributed behaviors to ADHD that at first glance seem to bear little connection with its DSM-5-TR criteria. Youth have indicated that the current diagnostic criteria of ADHD do not fully capture their experiences with the diagnosis, since these criteria were originally developed with children in mind (Ginapp et al., 2023; Zalsman and Shilton, 2016). By redefining ADHD, the video creators may seek recognition and acceptance for their currently undiagnosed behavior. To address these experiences in their videos, they adopt new terms such as 'ADHD paralysis' and 'rejection sensitive dysphoria' to effectively capture the problems they believe are part of ADHD in adolescence and adulthood. Youth also referenced 'executive dysfunctioning' in their videos to refer to regulation problems in cognitive processes, which is a term that has quickly gained popularity to explain ADHD behaviors (Roberts et al., 2017). In a similar vein, they indicated that people with ADHD experience hyperfocus, thereby implying that ADHD is not merely an attention deficit, as its name suggests, but rather an attention regulation issue.

Research suggests that youth with ADHD indeed are more likely to experience some of the behaviors they attribute to this diagnosis, such as hyperfocus, compared to undiagnosed individuals (Hupfeld et al., 2019). However, this does not imply that these behaviors are exclusive to people with ADHD. While some experiences depicted in the videos, such as feeling overwhelmed, could be precursors to behaviors listed in the DSM-5-TR, like having difficulty organizing tasks, this does not entail that all individuals who quickly feel overwhelmed should be diagnosed with ADHD. Many of the behaviors that youth attributed to ADHD in their videos are common experiences, also among those without a diagnosis. Moreover, the wide range of behaviors associated with ADHD on TikTok, including contradictory behaviors such as getting bored by repetitive tasks and enjoying repetition, makes it likely that most individuals will recognize themselves in at least some of the ADHD content. By attributing behaviors to ADHD that do not correspond with its diagnostic criteria, video creators contribute to bottom-up psychiatrization. Although ADHD is merely a name for a set of behavioral criteria, it is frequently understood as a chronic disease entity that causes problems (Te Meerman et al., 2022). This also became apparent in the way youth spoke about ADHD in their videos. They treat ADHD as a condition that one can 'have', and imply in their personal videos that ADHD can express itself differently in different individuals. This reification allows room for them to attribute other behaviors to ADHD that they believe might also be 'symptoms' of this underlying cause. As youth further expand the ADHD concept, a wider variety of behaviors are understood through this pathological lens. Consequently, viewers who self-identify with this content may come to see ADHD as a (medical) explanation for their behaviors, which was also reflected in the comments.

The current study shows that individuals frequently recognize themselves in TikTok content concerning ADHD, and may self-identify with this diagnosis as a result. Moreover, TikTok videos are also viewed by those with already existing (self-)diagnoses, who hope to gain further recognition and support for their issues (Eagle and Ringland, 2023). Research on autism videos showed that both diagnosed and undiagnosed youth redefine their sense of self in light of the diagnostic

information on TikTok (Alper et al., 2023). When they view their behavior as the result of a chronic brain disorder, this can negatively impact their self-image and cause feelings of hopelessness (Harari et al., 2023; Ching and Ma, 2021). Moreover, undiagnosed individuals may seek professional help after watching the videos, therefore further increasing pressure on the health care system. This may include youth who exhibit ADHD behaviors but do not experience impairment, as videos that provided accurate information often failed to mention that the described behaviors must impair functioning in order to qualify for diagnosis. After watching TikTok videos, these youth may self-diagnose when they have only mild problems. When they enter care, this could divert resources away from those experiencing more severe issues.

4.1. Limitations and future directions

Although the authors ultimately achieved high interrater reliability in the coding process, this was only accomplished after extensive discussion regarding the interpretation of both the depicted behavior and the criteria as outlined in the DSM. Given the lack of specificity of the DSM-criteria, additional guidelines needed to be agreed upon to decipher between DSM and non-DSM behaviors. While necessary, this process may have introduced bias, potentially impacting the study's findings. At the same time, this approach mirrors clinical practice, where professionals also have to navigate the ambiguity of this diagnosis by making subjective judgments. The categorization of non-DSM characteristics similarly required an inductive analysis of the data. As most depicted experiences did not fit neatly into one specific domain, it should be noted that alternative categorizations were possible. For example, experiences such as feeling overwhelmed or being sensitive to rejection, which were classified respectively as cognitive and social relation issues in this study, could also be interpreted as mood-related problems. These challenges in the coding process further highlight the complexity of mental health-related problems, and the difficulty to classify these experiences in a valid and clinically useful manner.

In the current study, we analyzed the 100 most popular videos mentioning characteristics of ADHD. These videos were included as their broad reach of over 316 million views means they likely impact a large audience. However, our findings might not be generalizable to less popular videos. False claims typically receive more attention on social media since they are novel and surprising, meaning less popular videos might provide more accurate information about ADHD (Vosoughi et al., 2018; Ceylan et al., 2023). Future research could benefit from using web scraping tools to systematically collect a larger dataset, which could provide a more complete understanding of how youth portray ADHD (Kanthawala et al., 2022). Furthermore, the current analysis was limited to the top comments, which were mostly very supportive of the content of the videos. It is unknown whether these comments are representative of the views of the wider audience, as comments may also be popular because they are interesting or surprising, rather than relatable. More nuanced expressed opinions might receive less engagement. Moreover, those who are critical of the video content might withhold from commenting all together in fear of rejection, meaning that their views might not have been adequately captured. To gain a deeper understanding of how youth are impacted by TikTok videos on ADHD, interviews or focus groups in which the experiences of both diagnosed and undiagnosed youth are collected could be valuable. Moreover, the broader societal implications of these videos could be evaluated if health care professionals track the number of self-diagnosed individuals that enter their care after engaging with TikTok content.

Future research could additionally explore the motives that youth hold for spreading misinformation on TikTok. Even though the current study focused on ADHD, misinformation about other diagnoses such as autism appears to be equally prevalent (Aragon-Guevara et al., 2023). While research suggests that youth may label previously undiagnosed behaviors in order to gain acceptance for their issues, platforms such as TikTok may also be used to change the deficit-based image of disorders

(Alper et al., 2023). In the current study, a small group of video creators attributed positive traits to ADHD in their TikTok videos, describing people with this diagnosis as creative, intelligent and charismatic. By emphasizing these positive traits, youth may attempt to improve the public image of ADHD, and, more indirectly, their own self-concept. This may allow them to experience the benefits of a diagnostic label, such as reduced feelings of guilt and a sense of understanding, while at the same time mitigating the negative effects that the diagnosis may have on their self-image.

4.2. Recommendations for practice

The current study shows that ADHD is often misrepresented on TikTok. Health professionals need to be made aware of the misinformation that is circulating online, to be able to better inform youth about what a diagnosis is, and perhaps more importantly, what it is not. In our study, we excluded videos created by health care professionals and researchers in order to focus on bottom-up psychiatrization. While it has been suggested that their content could help counter the stream of misinformation, these videos often receive much less traction on TikTok than those from non-professionals (Yeung et al., 2022). Besides, the high-paced nature of TikTok and its algorithm might also make it difficult to present a complete and nuanced image of ADHD on this platform. Instead, health professionals, teachers and parents are encouraged to adopt a normalizing approach to challenging behavior of children early on. The Stepped Diagnosis approach aims to avoid diagnoses whenever possible, by teaching those in the environment of children how to effectively manage these behaviors (Batstra et al., 2014). Later on, this might prevent adolescents and young adults from feeling the need to seek a diagnosis for behaviors that are perceived as challenging but are often a normal part of life.

Youth value that social media provides them with the opportunity to share and seek information about mental health on their own terms.

Therefore, rather than discouraging them from using the platform, it is recommended that youth are equipped with the skills necessary to successfully navigate this social media content. The current study showed that top-commenters accepted or even validated the information presented online, despite the inaccuracy and humorous undertone of most videos. Only a small group of these commenters rejected or at least questioned the diagnostic claims made on TikTok. Since our findings show that misinformation and correct information are often presented together and are therefore hard to dissect, it is crucial to teach youth to adopt a critical stance towards all mental health information shared on social media. Moreover, they should be made aware of how the intricate functions underlying these platforms determine what (biased) information they are exposed to. This would enable youth to use social media to their advantage, while minimizing potential harms to their self-image.

CRediT authorship contribution statement

Wietske de Vries: Writing – review & editing, Project administration, Investigation, Writing – original draft, Methodology, Formal analysis, Conceptualization. **Laura Batstra:** Supervision, Formal analysis, Writing – review & editing, Funding acquisition, Conceptualization. **Arjen van Assen:** Supervision, Writing – review & editing, Conceptualization.

Declaration of competing interest

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Appendix A. Transcription symbols

During transcription, symbols from Jefferson's notation were used to indicate speech characteristics that were relevant to this study (2008). See Appendix B-1 for an example of a fictional transcript in which these symbols are applied.

Table 3
Transcription Symbols

Symbol	Meaning
°word°	Quiet speech
WORD	Loud speech
>word<	Quick speech
<word>	Slow speech
word-	Talk is cut-off
wo:rd	Elongation of word
[word]	Talk overlaps
word = / = word	Latching talk
word	Action
¹ word	Timepoint in talk where action occurs
(O)	Commentary/interpretation of what is said

Appendix B. Fictional example of transcripts

Figure B-1 and B-2 are fictional examples of transcripts, which are compiled from multiple real included transcripts of videos. Fictional examples are provided to protect the privacy of the video creators.

Text	Spoken audio	Background audio	Non-verbal actions	Setting
POV: ADHD	One two one two=	((singing))	<i>Feet are shown, walking on a path</i>	In a park
	¹ Look at those leaves ²	((singing))	¹ Camera moves to side, shows trees ² Hand is shown taking a leave from a tree and squeezing it	
	I wonder what-	((singing))	<i>Feet are shown, walking</i>	
	³ This feels nice:		³ Hand is shown touching a fence while walking	
	How long have I-	((singing))	<i>Feet are shown, walking</i> ⁴ Hand is shown picking up a clump of dirt	
	[I need] to pick it up ⁴ and throw it ⁵		⁵ Hand throws clump of dirt on the ground	
	Ah: I am going to be late- ⁶	((singing))	⁶ Camera moves to side to show swing set	
	⁷ Wieh:: wieh::		⁷ Feet are shown, swinging on the swingset	

Figure B-1. Fictional Example of a TikTok Video Illustrating the Point of View (POV) of Someone with ADHD

Text	Spoken audio	Non-verbal actions	Setting
If you do these things you probably have ADHD			In living room
Random dance breaks		<i>Dances</i>	
Walking like this		<i>Walks around with arms swinging and knees slightly outwards</i>	
Making random noises	AH:::		
Pulling silly faces		<i>Squints eyes Wrinkles nose Sticks out tongue</i>	

Figure B-2. Fictional Example of a TikTok Video Attributing Behavior Regulation Issues to ADHD

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